

ASCII MASTER FLOW – PANEL 1/12: Epidemiological status ladder

OUTBREAK -> local excess over expected

EPIDEMIC -> population / regional excess

PANDEMIC -> sustained multi-country / continental spread

ENDEMIC -> expected persistent presence

ASCII MASTER FLOW – PANEL 2/12: Emergency-status firewall

DOMESTIC STATUS -> national / subnational legal basis

PHEIC -> IHR international-concern status

PANDEMIC EMERGENCY -> amended-IHR higher alert

WHO COORDINATES -> STATES RETAIN SOVEREIGN AUTHORITY

ASCII MASTER FLOW – PANEL 3/12: Risk-assessment map

AGENT / HAZARD + TRANSMISSION ROUTE

EXPOSURE + VULNERABLE GROUPS

SEVERITY + HEALTH-SYSTEM CAPACITY

UNCERTAINTY + PLACE / TIME -> PROPORTIONATE ACTION

ASCII MASTER FLOW – PANEL 4/12: Surveillance-to-action rail

- 1 SIGNAL / S-P-L REPORTING
- 2 ANALYSE TREND AND VERIFY
- 3 RAPID RESPONSE INVESTIGATION
- 4 CONTROL + FEEDBACK TO SURVEILLANCE

ASCII MASTER FLOW – PANEL 5/12: One Health triangle

HUMAN HEALTH <-> ANIMAL HEALTH

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ENVIRONMENT

JOINT SURVEILLANCE / INVESTIGATION; SECTOR OWNERS REMAIN

ASCII MASTER FLOW – PANEL 6/12: Preparedness portfolio

PREVENT -> WASH vector control infection prevention immunisation

PREPARE -> workforce labs stocks plans training drills

COMMUNICATE -> trusted accessible channels

CONTINUE -> routine health and social services

ASCII MASTER FLOW – PANEL 7/12: Outbreak-response loop

VERIFY -> INVESTIGATE -> CASE FIND / TEST
CARE + ISOLATE WHERE INDICATED
MANAGE EXPOSURE / COMMUNITY RISK
REASSESS DATA EFFECTS EQUITY AND PROPORTIONALITY

ASCII MASTER FLOW – PANEL 8/12: Quarantine-isolation split

QUARANTINE -> exposed, not known infected

ISOLATION -> infected / ill case

BOTH -> lawful proportionate supported and time-bounded

TRAP -> LABEL DOES NOT REPLACE CARE OR RIGHTS

ASCII MASTER FLOW – PANEL 9/12: Surge and continuity balance

SURGE -> staff space lab supplies referral transport

WORKFORCE -> protection rest communication support

ESSENTIAL CARE -> maternal child chronic emergency services

OUTCOME -> outbreak care without avoidable indirect harm

ASCII MASTER FLOW – PANEL 10/12: Risk communication chain

KNOWN + UNKNOWN + CHANGING

WHO / WHERE / WHAT ACTION / WHERE CARE

MULTILINGUAL ACCESSIBLE TRUSTED MESSENGERS

MONITOR RUMOUR STIGMA FEEDBACK AND CORRECT

MUST REMEMBER: Public-health emergency management joins surveillance, risk assessment,...

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ASCII MASTER FLOW – PANEL 11/12: Institutional map

MOHFW / NCDC-IDSP / ICMR -> STANDARD SURVEILLANCE EVIDENCE

STATE / DISTRICT / LOCAL -> PUBLIC HEALTH DELIVERY

DM AUTHORITIES -> CROSS-SECTOR COORDINATION

WHO / IHR -> INTERNATIONAL NOTIFICATION AND COOPERATION

CLOSE DISTINCTION: Case is not outbreak; outbreak is not automatically epidemic or...

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ASCII MASTER FLOW – PANEL 12/12: Public-health answer spine

DEFINE STATUS -> ASSESS ROUTE EXPOSURE SEVERITY CAPACITY

SURVEIL CONFIRM INVESTIGATE RESPOND AND REASSESS

ADD ONE HEALTH SURGE CONTINUITY COMMUNICATION AND EQUITY

ASSIGN FEDERAL / INTERNATIONAL ROLES + VERIFY OUTCOMES

EVIDENCE LIMIT: MECHANISM / STATUS / CAUSATION: Preserve disease/event definition,...

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